

An Autistic Teenager You Support

For support workers, PAs, mentors & youth workers — supporting an autistic teenager (roughly ages 12–18).

The one idea

Autism, through **monotropism**, means a teen's attention locks into a few deep, intense "tunnels." This is the **peak age for anxiety, depression and burnout**, and many autistic teens mask all day and arrive with you depleted. Your role is steady, predictable, low-demand support: keep routines, protect their interests, and watch for the overload and burnout they may not name. Take their inner life seriously, and never read a shutdown as "won't engage."

What you may see — and what's really going on

You may see...	What's often happening
Exhausted, withdrawn, refusing to take part	Probable burnout or overwhelm — distinct from laziness
Goes silent or leaves suddenly	Shutdown — involuntary; needs calm and time, not pressure
Clings to routines, objects, one interest	These are regulation and identity , not "obsessions"
Won't report pain, hunger or being unwell	Interoception differences; "not saying" ≠ "not suffering"
Wants things the same way, every time	Predictability lowers an already-high load

Try this

- **Keep staff, routines and places consistent;** warn before every change; co-create the plan *with* them.
- **Protect the interest and downtime** as regulation and recovery, not "indulgence."
- **Communicate literally, one thing at a time**, often in writing/text; allow processing time and choice.
- **Lower the sensory load** — quiet, low-light space; allow headphones, stimming and movement.
- **Have a calm meltdown/shutdown plan:** reduce input, ensure safety, give time, no restraint.
- **Ask directly, literally** about mood and safety if you're worried — and pass concerns on.

Avoid this

- Surprise changes, pressure to mask, or "join in!" demands when they're depleted.
- Stopping stimming, or reading a shutdown as refusal.
- Equating a calm, masked hour with being "fine."

When to get more support

Watch for **autistic burnout** (exhaustion, loss of skills, reduced tolerance) and signs of crisis — share concerns with the family and, where appropriate, the safeguarding lead. Investigate **pain or illness first** for any new distress. Signpost autism-led peer groups and mentors.

If the teen is a girl or marginalised gender

Camouflaging peaks here; the quiet, “anxious perfectionist” may be an exhausted autistic teen running on empty. Take a private, literal check-in seriously.

About this guide

*AI-assisted, derived from this project's research drafts — the **Family guide** (Part 4), the **Lifespan Practitioner & Health-Care guide** (Part 3.3) and the **Monotropism** brief. Uses identity-first language. **Informational — not a diagnostic tool**. Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020, burnout); Pearson & Rose (2021, masking); Kelly Mahler (2024). Full citations are in the source drafts.